

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	LES for the administration and monitoring of testosterone therapy in male hypogonadism (Testosterone undecanoate)
Commissioner Lead	Nicola Jones, Deputy Director for Primary Care
Provider Lead	
Period	1 st April 2025 – 31 st March 2026 (3 months' notice will be required to terminate this contract by either party)
Date of Review	

1. Population Needs

1.1 National/local context and evidence base

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	✓
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	✓
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	✓
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	✓
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	✓

Local Defined Outcome

Patients are provided with a service in line with the locally agreed pathway and standards of care. The service delivered will be subject to one clinical audit per annum and annual review of the

commissioned services in general practice, the structure of which will be developed collaboratively with the LMC.

3. Scope

3.1 Aims and objectives of service

Male hypogonadism is diagnosed by low serum testosterone levels in the presence of symptoms. Low testosterone in men can manifest with various symptoms, including decreased libido, erectile dysfunction, fatigue, loss of muscle mass, increased body fat, and mood changes like depression or irritability. Diagnosis is usually made in secondary care by endocrinologists and treatment involves testosterone replacement, either parentally (via injection) or topically. Testosterone cannot be taken orally as the hormone is broken down in the digestive tract and insufficiently absorbed.

The aim of this LES is to ensure patients requiring injectable testosterone for male hypogonadism have access to the provision of this within general practice.

This LES refers only to testosterone undecanoate and the brand Nebido, all mentions of testosterone should be interpreted as testosterone undecanoate. The LES does not cover any other testosterone salt, or any other brand of testosterone.

3.2 Service description/care pathway

This Local Enhanced Service is for the management of cis men registered with Devon practices who have had a diagnosis of hypogonadism made in NHS secondary care by a specialist in endocrinology or an appropriately qualified clinician using test results from an accredited laboratory. It recognises that the prescribing and monitoring of testosterone therapy in this cohort of patients is beyond the scope of the core GMS contract for GP providers.

This Local Enhanced Service is for injectable testosterone undecanoate (Nebido brand)

The dose and brand of testosterone will be defined by the endocrinologist alongside the duration of therapy and the process for de-prescribing.

It is important to monitor the appropriateness of testosterone treatment and development of side effects whilst eligible patients are on testosterone treatment. Usually, once established on testosterone treatment they will be monitored annually if no adverse effects are identified.

For detailed product information, including monitoring guidelines, please refer to the British National Formulary (BNF), the product's Summary of Product Characteristics (SPC), and the formulary. Consulting these sources will ensure you have the most current and accurate information, as clinical guidance is subject to change.

3.3 Population covered

Cis men with confirmed hypogonadism (usually 2 early morning serum samples in the presence of symptoms).

Hypogonadism may be primary (testicular) or secondary (due to issues elsewhere in the hormonal axis). The management plan, dose titration and duration of therapy would usually be defined by the

specialist in secondary care who should be available for advice regarding management if issues arise following the commencement of therapy.

3.4 Any acceptance and exclusion criteria and thresholds

Exclusions include female patients requiring testosterone as part of specialist menopause management.

Patients who have not received appropriate NHS diagnosis from an endocrinologist or an appropriately qualified clinician using test results from an accredited laboratory.

Patient Safety:

Any patient safety incidents relating to the injection of testosterone to be reported through the agreed ICS incident reporting process.

Consent:

Practices will be required to ensure that Informed consent is sought – this may be in the form of implicit consent (where patient chooses the Service).

Prescribing:

Participating practices must ensure prescribing is in line with NICE guidance and national recommendations and should comply with the Joint Formulary. Please refer to correct Formulary for your area as guidance regarding Nebido differs.

Training and accreditation:

Clinicians administering testosterone therapy should be appropriately trained in injection techniques. The clinician administering testosterone therapy can demonstrate they have undertaken CPD on Male Hypogonadism and treatment. For product information, to include information on monitoring, please see the BNF, the formulary, and the product's summary of product characteristics (SPC).

[Nebido 1000 mg/4 ml solution for injection - Risk Management Materials - \(emc\) | 14631](#)

3.5 Interdependence with other services/providers

Per guidance in Devon Formulary, ICB guidance regarding shared care and other guidance in place e.g. regarding the primary care - secondary care interface.

4. Applicable Service Standards

4.1 Applicable national standards (eg NICE)

NICE guidance where applicable.

4.2 Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

Not applicable

4.3 Applicable local standards

Refer to Devon Joint Formulary (South & West and North & East).

Please note there is different guidance regarding Nebido across each formulary.

[6.4.2 Male sex hormones and antagonists - South & West](#)

[6.4.2 Male sex hormones and antagonists - North & East](#)

Equality and Diversity:

The service must conform to legislation prohibiting discrimination and strive to achieve good practice regarding equality, diversity and inclusion.

Complaints and commendations:

Participating practices are required to establish and operate a complaints and commendations procedure in line with NHS guidelines to deal with any complaints in relation to any matter connected with the provision of services. All complaints should be monitored, audited and appropriate action taken when required.

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each practice must ensure that any personnel involved in providing any aspect of care under this scheme has the necessary training, skill and competency to do so. A Registered Nurse may delegate care delivered by Nursing Associates and Healthcare Assistants as appropriate.

[Delegating medicines administration - Care Quality Commission](#)

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable

6. Location of Provider Premises

The provider will provide this service from any location as included in the practice's GMS, PMS or APMS contract where these facilities enable the provider to ensure privacy and dignity and appropriate infection control.

7. Funding for 2025-2026

NHS Devon will pay a fee of £13.87 per injection per patient to the provider on receipt of claims made using the current agreed method for LES claims.

This payment covers the item of service of the injection, for prescribing and administering. Plus, an annual fee of £62.22 for the annual monitoring of the patient, once the patient is stable

Post payment verification of claims may be undertaken from time to time in line with guidance and relevant policy.

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.